

OCTOBER 2026

WHAT WE HAVE BUILT AND WHAT COMES NEXT

An update on Tojo, our hospital operations partner: the product so far, the dates to launch, and how we take it to market.

INVESTOR UPDATE · OCTOBER 2026

TOJO

HOSPITAL EXPERIENCE, TURNED INTO SOFTWARE

What we have built, what is left, and our plan to reach hospitals from January 2027.



PAID LAUNCH

01.01
2027

THE ROAD TO LAUNCH

Where Tojo stands today, and the four steps between now and paying hospitals.

END OCT

Chat ready for daily use

MID NOV

Free trials start at hospitals

MID DEC

Automation suites ready

JAN 2027

We go to market

02

CHAPTER TWO

WHY TOJO IS DIFFERENT

Where hospital software, dashboards and general AI stop short

How Tojo thinks ahead, shown on real screens

Why getting this right took one more month

WHY TOJO IS DIFFERENT

*Text books are not the same as years of
running a hospital.*

This chapter shows, on real Tojo screens, what
changes when software is built from experience.

CONTENTS

01 Where we stand

02 Why Tojo is different

03 What we build next

04 Going to market

05 The numbers

02 WHY TOJO IS DIFFERENT

Most software waits for a question and gives back a page of text. Tojo maps the hospital, works out the real cause, and shows the next steps before he is asked. This chapter shows how, and why it took one more month to get right.

WHAT IS BUILT, AND WHAT IS LEFT

DONE

THE CHAT ENGINE

The back end that runs every conversation with Tojo.

TOJO, TRAINED

On playbooks written from years of running hospitals, by people who ran them.

THE TOOLS

Diagnosis, Solutions, Processes and Automations. Each one lets a user go deeper with Tojo.

SCREENS

Desktop and phone, worked end to end for Discharge and Bed Management.

UNDER WAY · END OCTOBER

FINISHING THE CHAT

Ready for daily use by hospital teams.

MORE DEPARTMENTS

OPD & Diagnostics and Supply Chain conversations brought to the same level.

TEAM TESTING

Our hospital experts run real cases through Tojo before any hospital sees it.

NEXT · MID DECEMBER

AUTOMATION SUITES

Discharge, Bed Management, OPD & Diagnostics, and Supply Chain, from medicines and consumables to large equipment purchases.

WORKS WITH THEIR SOFTWARE

The suites either sit next to a hospital's own software or replace parts of it.

TEAM Development and AI teams, now joined by hospital software (ERP) experts.

ONE SET OF TOOLS, TWO JOBS

The tools we have built are not only for talking. The automation suites will use the same tools to do the work.



UNDERNEATH · BUILT

Tojo's knowledge: how a hospital really runs, department by department.

ON TOP · APRIL 2027

Simulation and whole-hospital monitoring.

DEPARTMENT BY DEPARTMENT

● Done ◐ Under way ○ Planned

	TOJO KNOWS IT	CHAT AND TOOLS	AUTOMATION SUITE
Discharge process Patient ready to bed free again	●	●	◐ Six automations designed
Bed management Admissions, stay, turnover	●	●	◐ Night plan engine designed
OPD & diagnostics Consult to test to report	◐	◐	○
Supply chain Medicines, consumables, large purchases	●	◐	○

All four suites to be ready by mid December 2026.

HE MAPS THE PROBLEM BEFORE HE ANSWERS IT



A hospital asks: "Patients are fit by mid-morning but leave late afternoon."

- 1 THE WHOLE ROAD, FIRST**
Tojo does not reply with a paragraph. He draws all eleven stops a patient passes on the way home, each run by a different team.
- 2 WHAT EXPERIENCE KNOWS**
His note: most delays start long before the billing desk. A text book blames billing. People who ran hospitals know better.
- 3 THE NEXT QUESTION IS READY**
The user taps to confirm, reject or go deeper. No typing, no prompt writing.

He asked about billing. Tojo showed him the doctors' round.

HE FINDS THE REAL PROBLEM, NOT THE STATED ONE

A TYPICAL GENERAL AI REPLY · ILLUSTRATION

Here are ten ways to reduce discharge delays in hospitals:

1. Plan discharge from the day of admission.
2. Use a standard discharge checklist.
3. Improve communication between teams.
4. Set a target discharge time, such as 11 AM.
5. Use technology to track patient flow.
6. Hold daily huddles with nursing staff.
7. Speed up billing and insurance approvals.
8. Arrange transport in advance.
9. Train staff on best practice.
10. Measure and review regularly.

TOJO, FOR THE SAME HOSPITAL

RESOURCES

DIAGNOSIS

SOLUTIONS

AUTOMATIONS

PROCESSES

DISCHARGE PROCESS

IN CONVERSATION WITH TOJO

Guided tour

YOUR TASKS FOR THE DAY

WHAT IS REALLY WRONG

UNUSED HOURS, AND NOBODY IN CHARGE

Doctor decides
7 PM

TIME UNUSED
Nothing happens overnight
16 HOURS

Doctor signs
11 AM

Papers and bill made

Patient goes home
ABOUT 3 PM

Ready by 10 AM, home by 3 PM: about 5 hours of waiting. All times are yours.

Beds in use
Over 80% needs one person in charge

You: 88%

80%

Patients going home each day
Over 40 a day needs the same

40 to 45

40

Patients going home each day
FROM YOUR NUMBERS

42.5

Hours each one waits to leave
YOUR NUMBER

5 HOURS

What one bed earns in a day
YOUR NUMBER

₹32,000

Hours beds sit waiting each day

213

Full days of bed time lost each year

3,232

Money those beds could earn the most it could be, not a promise

₹10.3 CRORE

True only if each freed bed gets a new patient that day. 42.5 is the middle of your 40 to 45.

T TOJO

Tojo · 250-bed hospital, Bhubaneswar

@Point1 @Point2

One bed earns about ₹32,000 a day. Most patients are ready by 10 and leave around 3, so they wait about 5 hours. The doctors usually decide who's going home on the 7 PM round, but they don't sign till morning.

2

Your real problem isn't slow discharge. It's time nobody uses, and nobody being in charge.

Here's why. Your doctors already decide in the evening. But nothing happens until they sign the next morning. Most of the going-home work could be done overnight. Nobody does it today because:

- Nothing is automatic, so starting early means doing the work twice.
- No team moves until the doctor signs.
- Nobody has ever been asked to use the evening.

The second reason: your own numbers show you now need one person in charge of discharge. Does this match what you see on the wards?

TOJO'S NOTE

The time is already there. Nobody uses it.

ADD TO A POINT — PICK ONE OR MORE

P1

The hours nobody uses overnight

P2

Papers and bill made only after signing

- 1 Doctors decide at 7 PM. Nothing happens for 16 hours.
- 2 “Your real problem isn’t slow discharge. It’s time nobody uses.”
- 3 A price on it, from their own numbers: ₹10.3 crore a year.

CORRECT. GENERAL. NOTHING TO ACT ON TOMORROW.

HE PLANS FOR PEOPLE, NOT ONLY FOR PROCESS

APPROVER

Head of Operations

Approves: The new order of steps, and splitting discharges from admissions.

If this never comes: the first busy morning sends everyone back to the old order.

Show me the people who must agree

Several teams touch discharge, but only a few are asked to change much. One person has to approve it. Why each matters:

- Doctors are asked for no extra time. They already decide at 7 PM, which is why they can agree.
- Typists have the most to give up, so they check drafts first, and nobody loses a job at the start.
- The Head of Operations must approve the new order, or it won't last past the first busy morning.

Next is who is in charge: whether you need anyone new. Shall we go there?

TOJO'S NOTE

"If this never comes, the first busy morning sends everyone back."



The user only asked to see who must agree. Tojo answered and went further.

WHAT HE SAW WITHOUT BEING ASKED

Typists have the most to give up, so their work changes first and no one loses a job at the start.

One person must approve the new order, or it will not last past the first busy morning.

Doctors are asked for no extra time, which is why they will agree.

ONLY SOMEONE WHO HAS RUN A HOSPITAL PLANS FOR THIS.

FOUR KINDS OF HELP, SIDE BY SIDE

	Hospital software ERP	Dashboards BI	General AI CHAT ASSISTANTS	TOJO
What it does	Keeps records	Shows charts of the past	Answers questions in text	Finds the cause and shows the fix
Where its knowledge comes from	What staff type in	What staff type in	Books and websites	Years of running hospitals
What you get back	Screens and forms	Graphs	Long paragraphs	Maps and flows you can tap
Does it think ahead?	No	No	Only when asked	Yes, it plans the next steps
Does it do the work?	Only what staff enter	No	No	Yes, with the automation suites

Tojo does not replace a hospital's records on day one. He works on top of them, and later takes over the parts that slow the hospital down.

ONE QUESTION, FOUR ANSWERS

THE QUESTION “Why do our patients leave so late in the day?”

HOSPITAL SOFTWARE

“Here are the discharge times, where staff entered them.”

A RECORD

DASHBOARD

“Average discharge time this month: 3:40 PM.”

A NUMBER

GENERAL AI

“Here are ten ways to reduce discharge delays...”

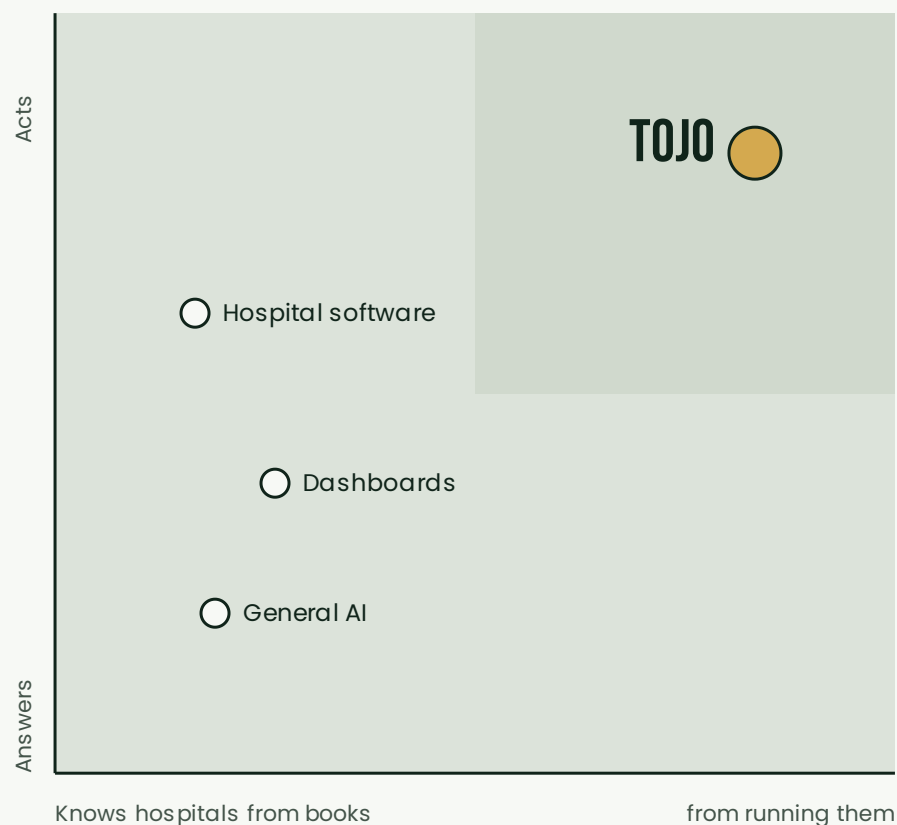
A LIST

TOJO

“Your doctors decide at 7 PM, then nothing happens for 16 hours, and nobody owns it. That costs you up to ₹10.3 crore a year. Here is the fix, who must agree, and a two-week trial on one ward.”

A PLAN

BUILT FROM EXPERIENCE, BUILT TO ACT



WHY THE CHAT IS ONE MONTH LATER

We aimed for mid October. We now aim for mid November. A general chatbot could have shipped sooner. Tojo needed three things it does not:

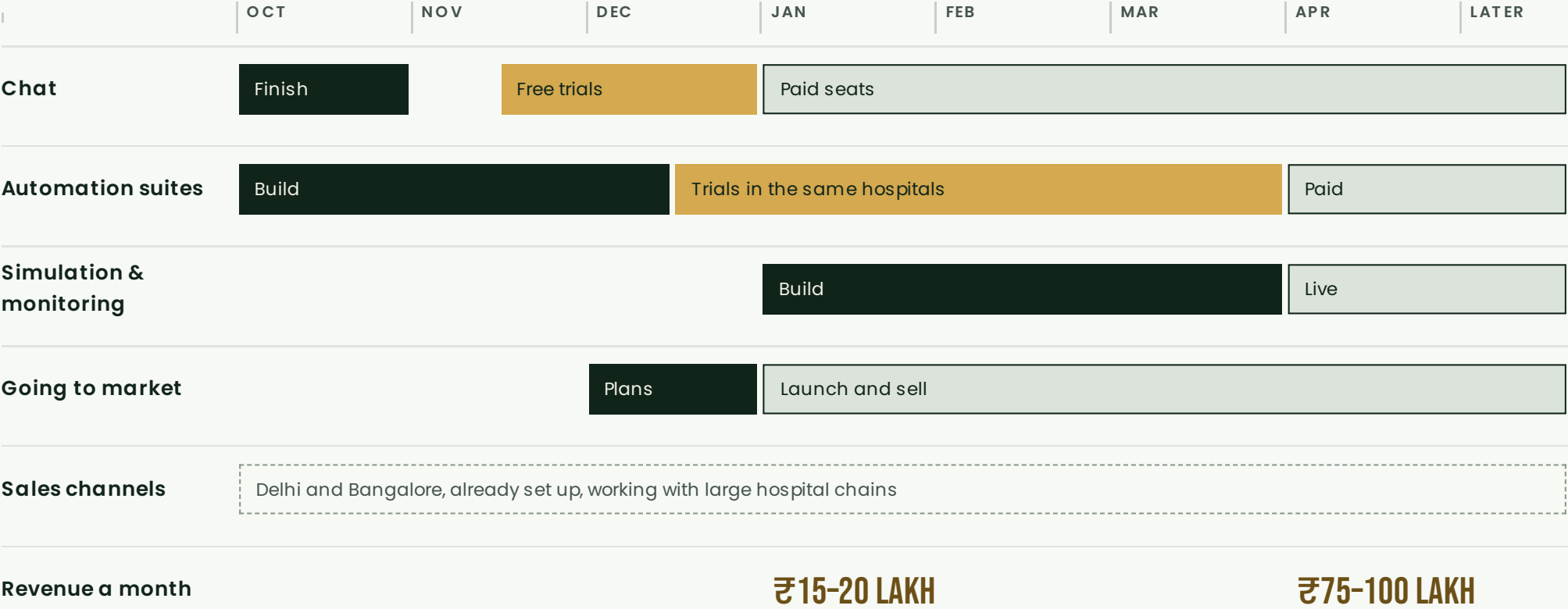
- 1 Teaching him how hospitals really run.** Department by department, from our own years of running them.
- 2 Tools that talk now and act later.** The same tools will drive the automation suites.
- 3 Answers drawn as maps.** Every reply is a screen the user can explore, not a page of text.

One extra month now saves the automation suites from starting over.

FROM TODAY TO THE END OF 2028



SIX MONTHS, MONTH BY MONTH



■ Build ■ Free trial ■ Live and paid

NOW, NEXT, THEN, LATER

NOW

OCT 2026

Chat finished by end of month

More departments brought up to level

ERP experts join the team

NEXT

NOV – DEC

Free trials at hospitals from mid November

Automation suites ready mid December

Launch and market entry plans in December

THEN

JAN – MAR 2027

Go to market from 1 January

6 to 10 hospitals, 3 to 4 seats each

₹15–20 lakh a month

LATER

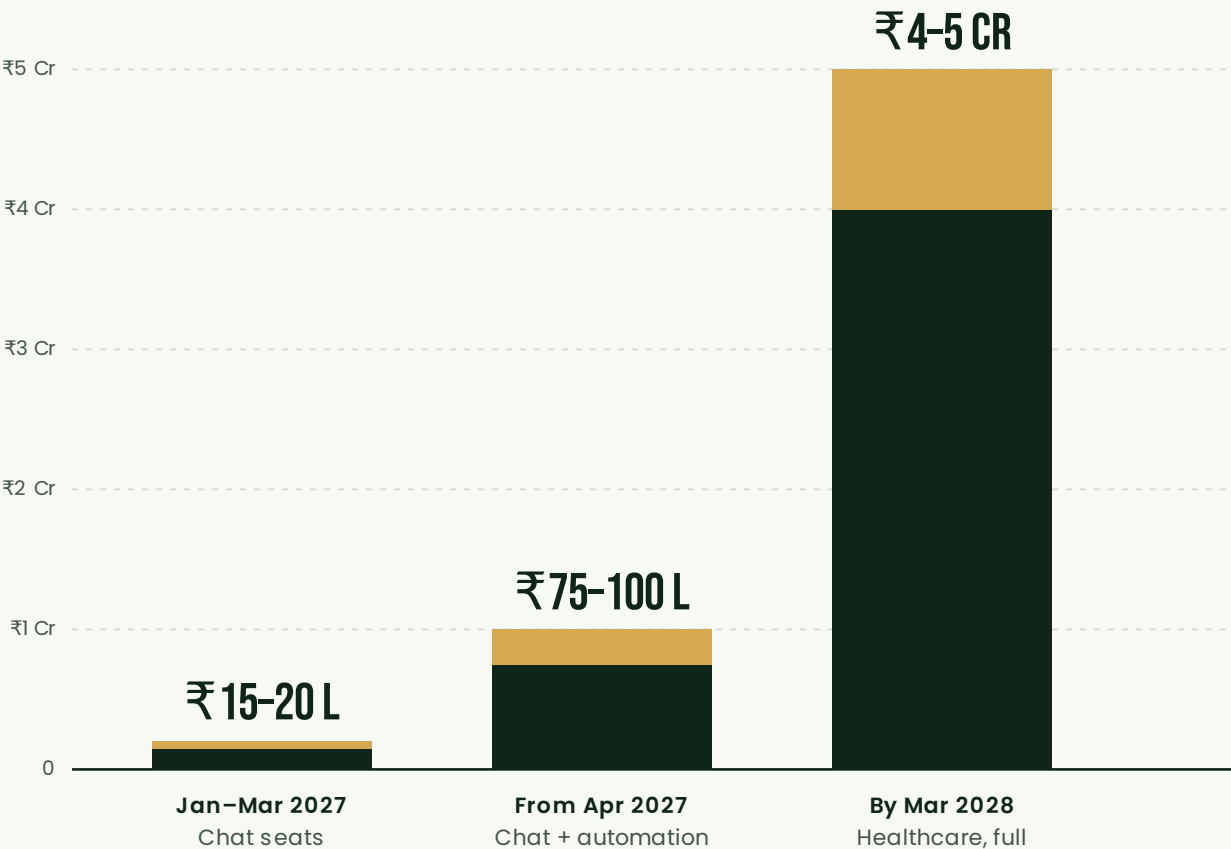
APR 2027 ON

Simulation and whole-hospital monitoring

₹75–100 lakh a month, then ₹4–5 crore by March 2028

New sectors by end 2028

REVENUE A MONTH, STAGE BY STAGE



END 2028 · ALL FOUR SECTORS

₹250 CR

a year in potential revenue

Healthcare plus hospitality, large retail and manufacturing, each at ₹3-5 crore a month. Too large to draw on the same scale.

Low case Up to high case

HOW EACH NUMBER IS MADE

JAN – MAR 2027 Chat only	6-10 hospitals	×	3-4 seats each	×	₹25-50K a seat a month	₹15-20 L a month
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FROM APR 2027 Chat, wider	20 hospitals	×	₹1L average a month			₹20 L
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Plus automation	10 of those hospitals	×	₹4-5L for one or more suites			₹40-50 L
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Total, with a push	Simulation and monitoring sales on top				₹75-100 L a month
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BY MAR 2028 Healthcare, full	More hospitals, more suites per hospital				₹4-5 CR a month
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END 2028 Four sectors	4 sectors	×	₹3-5 CR a month each			₹250 CR a year, potential
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FOUR NUMBERS TO REMEMBER

JAN-MAR 2027

₹15-20_L

a month from chat seats at 6 to 10 hospitals

FROM APR 2027

₹75-100_L

a month once automation suites are live

BY MAR 2028

₹4-5_{CR}

a month from healthcare alone

END 2028

₹250_{CR}

a year, with hospitality, retail and manufacturing

START WITH ONE PROBLEM. GROW ACROSS THE HOSPITAL.

STEP 1 · NOV 2026

FREE TRIAL

1 to 2 months of chat at no cost.
Tojo shows the hospital its own
hours and rupees lost.

STEP 2 · JAN 2027

PAID CHAT SEATS

3 to 4 seats for the people who
run the hospital day to day.
₹25,000 to ₹50,000 a seat.

STEP 3 · APR 2027

AUTOMATION SUITES

The same hospital switches on
suites for discharge, beds, OPD or
purchases. ₹4–5 lakh a month.

STEP 4 · 2027–28

THE WHOLE HOSPITAL

Simulation and full monitoring.
Chains roll Tojo out to every unit.

WHERE WE SELL, AND TO WHOM

WHERE

DELHI BANGALORE

Sales channels already set up in both cities, aimed at large hospital chains.

IN PLACE

WHO

WHO SIGNS

Chain CEO or COO

WHO USES TOJO

Heads of operations, medical superintendents, unit heads, finance heads

WHY THEY CARE

Beds, time and money lost every day, shown in their own numbers

HOW WE WIN

A free trial on one problem the hospital already feels.

Tojo puts a rupee figure on it in the first conversation.

Trial hospitals become the stories we tell the next chain.

DECEMBER 2026

Launch plan and market entry plan ready before we go to market on 1 January.

THE PLAN, PHASE BY PHASE

WHEN	WHO WE REACH	WHAT WE OFFER	HOW	SUCCESS LOOKS LIKE
NOV – DEC 2026 TRIALS	6 to 10 hospitals from our chain contacts	Chat, free for 1 to 2 months	Delhi and Bangalore channels, founder-led	Each hospital sees its own losses in rupees
JAN – MAR 2027 LAUNCH	Trial hospitals, then their sister units	Paid chat seats	Launch plan, trial stories, direct sales	₹15–20 lakh a month
APR 2027 ON GROW	5 to 10 more hospitals, 20 in all	Automation suites, simulation, monitoring	Upsell inside each hospital	₹75–100 lakh a month
2027 – 28 SCALE	More cities and chains	The whole hospital	Market entry plan, partners	₹4–5 crore a month by March 2028